

BIOSYNC CLINICAL LABORATORY REPORT — URGENT

Patient: Terrence L. Bouchard (DOB: 07/22/1976) | Date: April 29, 2026

Vital Signs & Biometric Readings

Heart Rate	116 BPM
Blood Pressure	162/108 mmHg
SpO2	86%
Temperature	103.8°F
Glucose	134 mg/dL
Weight	183 lbs
BMI	27.6
HRV	18 ms

Clinical Notes

Patient brought in by partner James Bouchard via emergency. Patient presenting with acute respiratory distress, high fever, productive cough with blood-tinged sputum, and confusion. Nevirapine and sulfa allergy documented — both avoided. Currently on Biktarvy (adherence inconsistent per partner), Escitalopram 40mg, Rosuvastatin 10mg, Amlodipine 5mg, Dapsone. Heart rate critically elevated at 116 BPM — tachycardia. Blood pressure 162/108 mmHg — hypertensive crisis. SpO2 critically low at 86% — supplemental oxygen via non-rebreather. Temperature 103.8°F — critical fever. Chest X-ray: bilateral interstitial infiltrates consistent with Pneumocystis jirovecii pneumonia (PCP) — most likely given CD4 of 312 and ART non-adherence. IV TMP-SMX contraindicated (sulfa allergy) — IV Pentamidine initiated. HRV 18 ms — severely depressed. Patient admitted to pulmonary unit.

Provider Notes

PCP pneumonia confirmed — life-threatening opportunistic infection in setting of HIV non-adherence and CD4 decline. IV Pentamidine 4mg/kg daily — monitoring for hypoglycemia and nephrotoxicity. High-flow oxygen — respiratory team managing. Corticosteroids added: Prednisone 40mg for PCP adjunctive therapy. Antihypertensive IV: Labetalol for crisis. Biktarvy to be restarted immediately — adherence support nurse assigned to patient. Psychiatry consulted — inpatient psychiatric evaluation requested. Resistance panel pending. Partner James Bouchard designated healthcare proxy with patient consent. Family notified. Patient condition critical but potentially reversible with treatment. ICU stepdown bed reserved. Prognosis dependent on treatment response. Full infectious disease team engaged. Ordering Provider: Dr. Michelle Tran, MD — URGENT. Facility: BioSync Clinical Health Center — Pulmonary Unit